

Women's Specialists of Clear Lake

Dr. Geoffrey H. Klein, MD · FACOG · DABOM · 400 W Medical Center Blvd, Suite 300, Webster, TX 77598 · (281) 557-0300



Gestational Diabetes (GDM)

Managing blood sugar during pregnancy to protect you and your baby

WHAT IS GDM?

Gestational diabetes is high blood sugar that develops during pregnancy in women who didn't have diabetes before. It occurs in **6–9% of pregnancies**. The placenta produces hormones that can block insulin's action (insulin resistance). When the pancreas can't keep up, blood glucose rises.

RISK FACTORS

- BMI ≥25 (overweight or obesity)
- Family history of type 2 diabetes
- Previous GDM or large baby (>9 lbs)
- Hispanic, Black, Asian, or Native American heritage
- Age over 35
- Polycystic ovary syndrome (PCOS)
- Prediabetes before pregnancy

BLOOD GLUCOSE GOALS

Timing	Target
Fasting (before breakfast)	≤95 mg/dL
1 hour after meal	≤140 mg/dL
2 hours after meal	≤120 mg/dL

Check blood sugar 4 times daily: fasting + after each meal. Log results and bring to every visit.

DIET MANAGEMENT

- Distribute carbs throughout 3 meals + 2–3 snacks
- Breakfast carbs limited (15–30g) — morning insulin resistance highest
- Choose complex carbs: whole grains, legumes, vegetables
- Avoid sugary drinks, juice, white bread, white rice
- Pair carbs with protein and healthy fat at every meal
- Work with a registered dietitian for meal planning

MEDICAL MANAGEMENT

- Diet + exercise controls GDM in ~70–80% of cases
- **Insulin** — first-line medication when needed **RX**
- **Metformin** — oral option, crosses placenta **RX**
- Weekly monitoring appointments once on medication
- Nonstress tests starting at 32–36 weeks
- Ultrasound to check fetal growth (possible macrosomia)
- Delivery planning based on glucose control

RISKS IF UNCONTROLLED

- **Baby:** Macrosomia (large baby), birth injury, low blood sugar at birth, jaundice, preterm birth, increased future diabetes risk
- **Mother:** C-section, preeclampsia, type 2 diabetes risk (50% within 10 years)
- GDM usually resolves after delivery
- Postpartum glucose test at 6–12 weeks required

⚠️ CALL YOUR DOCTOR IF YOU EXPERIENCE

- ⚠️ Fasting blood sugar consistently over 100 mg/dL
 - ⚠️ Severe headache or blurred vision
 - ⚠️ Decreased fetal movement
 - ⚠️ Nausea/vomiting preventing you from eating
- ⚠️ After-meal readings over 140–150 mg/dL
 - ⚠️ Excessive thirst or urination
 - ⚠️ Signs of low blood sugar: shakiness, sweating, confusion
 - ⚠️ Illness or infection (can spike blood sugar)

♥️ Gestational Diabetes Is Very Manageable

Most women with GDM have healthy pregnancies and healthy babies with the right care. You have significant control over your blood sugar through what you eat and how you move. Every healthy meal and every walk around the block is a gift to your baby. You are doing an amazing job — keep going!